

MeasureWise Sample Audit Binder

A branded preview of how MeasureWise organizes quality improvement documentation, evidence tracking, and leadership-ready reporting — updated for CY2026 HRSA UDS reporting requirements.

Organization	Riverbend Community Health Center (fictional)
Reporting Period	January 1, 2026 to March 31, 2026
Prepared In	MeasureWise sample format
Standards Alignment	HRSA Chapter 10 · NCQA PCMH Q-PASS · CY2026 UDS (per PAL 2025-05)
Document Version	v3.0 — Updated August 25, 2026
Prepared By	MeasureWise Product Demonstration Team

What this sample shows

MeasureWise is built to support HRSA quality-improvement documentation, PCMH evidence organization, and UDS performance monitoring — including the proposed CY2026 UDS measure and reporting changes released by HRSA in Program Assistance Letter 2025-05. This export uses fictional data to demonstrate how active improvement work, assigned tasks, documentation, and measure oversight can be consolidated into one leadership-ready, audit-ready packet.

What MeasureWise Automates

One-click evidence packets

Generate a print-ready binder for any date range, including cycle logs, task evidence, baseline-to-result deltas, lessons learned, and next-cycle linkages.

Guided PDSA methodology

Walk teams through Aim, Prediction, Measurement, Test, Analysis, and Decision with coaching prompts and reusable templates.

UDS measure dashboards

Track measure progress with trend analysis and run charts, kept current with each year's UDS Table 6A/6B/7 revisions and eCQM version updates.

PCMH evidence organization

Capture and organize recognition evidence mapped to standards, ownership, document dates, and review cadence.

Prospect Disclaimer: Demonstration only; does not represent a real clinic or live customer environment. Actual exports vary by configuration, features, and workflows. HRSA/PCMH/UDS alignment language refers to documentation support, not certification or compliance guarantee. CY2026 UDS content reflects proposed changes per HRSA PAL 2025-05 (Dec. 8, 2025); final requirements will be confirmed in the 2026 UDS Manual.

Executive Summary

During Q1 2026, Riverbend Community Health Center used a standardized quality-improvement operating workflow to monitor four priority UDS measures, run three active PDSA cycles, and assign evidence-collection work to named owners. Two high-priority follow-up items remain open, each with an assigned owner and due date. This export provides leadership with a dated record of performance review, improvement decisions, supporting evidence, and next-cycle actions — and confirms the organization's monitoring approach is aligned with HRSA's proposed CY2026 UDS reporting changes.

3

Active PDSA Cycles

Across 4 UDS measures

4/4

Measures Trending Favorably

vs. baseline, period-over-period

4

Open Evidence & Task Items

2 flagged high-priority

Needs Follow-up

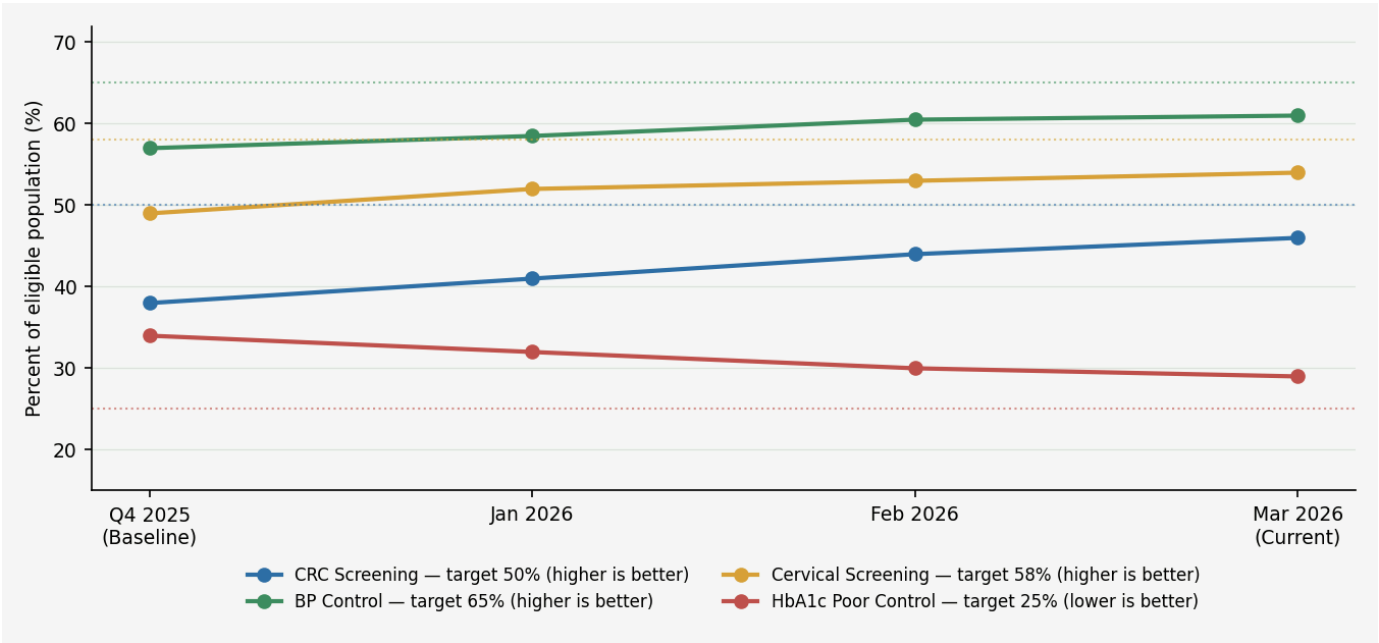
Audit Readiness Status

5 of 6 areas ready; 1 area pending task closure

Snapshot of Results

Area	Current Status	Operational Meaning
UDS measure review	Updated monthly	Leadership can review progress without rebuilding reports manually
PDSA documentation	Standardized	Improvement work is recorded consistently across projects
Evidence collection	Centralized, 6 of 6 items logged	Audit materials are easier to locate, date, and verify
Task ownership	Assigned by role	Follow-up work is visible and accountable, not informal
Review cadence	Active, weekly/monthly per function	Teams have a repeatable process, not a one-time cleanup effort
CY2026 UDS readiness	Tracking — see Section 9	Proposed measure and reporting-table changes are logged for the 2026 reporting year

UDS Measure Trends



Source: EHR quality dashboard exports, reporting period January 1 – March 31, 2026. Dotted lines indicate the Q2 2026 target for each measure. Directionality is labeled per measure because HbA1c poor control improves when the rate decreases, while the other three measures improve when the rate increases.

Quality Program Overview

Governance model

Each function has a defined role, cadence, and expected output so quality work does not live in a single person's inbox.

Function	Role	Cadence	Output
Quality oversight	Director of Quality Improvement	Monthly	Committee review and escalation
PDSA coordination	PCMH Coordinator	Biweekly	Project updates and action tracking
Clinical measure review	Clinical Operations Manager	Monthly	Trend monitoring and intervention prioritization
Evidence management	Compliance Lead	Ongoing	Audit-ready document organization
UDS regulatory tracking	Compliance Lead	As released by HRSA	Monitors HRSA PALs and UDS Manual updates for measure/table changes

2026

Standard Operating Flow

1. Identify a priority gap, risk area, or reporting need.
2. Launch or update a PDSA cycle with a documented aim and prediction.
3. Assign owners, deadlines, and intervention tasks.
4. Monitor trend data, process measures, and implementation progress.
5. Document the decision to adopt, adapt, or continue testing.
6. Export a structured, traceable binder for review, survey prep, or leadership oversight.

Measure Monitoring Summary

Measure	Baseline	Current	Target	Change	Direction
Colorectal Cancer Screening	38%	46%	50%	+8 pp	Higher is better
Cervical Cancer Screening	49%	54%	58%	+5 pp	Higher is better
Hypertension Control	57%	61%	65%	+4 pp	Higher is better
Diabetes: HbA1c Poor Control	34%	29%	25%	-5 pp	Lower is better

Interpretation: All four tracked measures moved in the favorable direction this period. Performance improvement was linked to more consistent outreach workflows, clearer task ownership, and stronger documentation reliability. See Section 9 for how these measures map to their CY2026 eCQM versions.

Active Improvement Portfolio

Project	Aim	Owner	Start Date	Stage	Decision
CRC Outreach Reliability	Increase screening completion from 38% to 50% in 90 days	PCMH Coordinator	2026-01-08	Study	Adapt
BP Recheck Workflow	Improve same-day BP recheck documentation from 62% to 90%	Nurse Manager	2026-02-01	Act	Adopt with revisions
Diabetes Lab Follow-Up	Reduce overdue follow-up calls older than 14 days by 40%	QI Analyst	2026-02-12	Study	Continue

Detailed PDSA Example — CRC Outreach Reliability
Test period: January 8 – March 20, 2026 · Reviewed at committee meeting March 18, 2026

Aim: Increase colorectal cancer screening completion from 38% to 50% among eligible adults within 90 days.

Problem: Eligible patients were identified inconsistently, and outreach activity was documented across multiple disconnected workflows.

Element	Documentation
Change idea	Build one outreach queue with clear MA ownership and weekly review
Prediction	A single workflow will improve completion and reduce missed outreach opportunities
Population	Adults aged 45–75 due for screening (CY2026 UDS Table 6A, Line 8, eCQM CMS130v14)
Process measure	Percent of eligible patients assigned to outreach queue within 48 hours
Outcome measure	CRC screening completion rate
Data source	Preventive screening list and outreach tracker (EHR export)
Review cadence	Weekly

Do
Launched one shared outreach queue, standard patient language, and a daily owner for status review.

Study
Completion improved in six weeks. Return rates varied by contact method and clinic site.

Act
Adapted the process with a day-10 text reminder and care coordination escalation after two failed attempts.

Linked evidence: Outreach workflow SOP v2.1 (EV-006) · Weekly queue report (EV-009) · QI committee minutes, March 18, 2026 (EV-002) · Staff training roster (EV-005)

Documentation and Evidence Tracking

Evidence Register

ID	Evidence Item	Purpose / Standard	Owner	Doc. Date	Status
EV-001	QI committee charter	Program oversight evidence	Director of QI	2025-11-02	Complete
EV-002	Quarterly committee minutes	Leadership review documentation	Executive Assistant	2026-03-18	Complete
EV-003	PDSA logs	Improvement methodology evidence	PCMH Coordinator	2026-03-20	Complete
EV-004	Measure trend export	Performance oversight evidence	QI Analyst	2026-03-31	Complete
EV-005	Staff retraining sign-in sheet	Education evidence — BP workflow	Nurse Manager	Pending	In progress
EV-006	Outreach workflow SOP	Standardized process evidence	Clinical Ops Manager	2026-01-05	Complete
EV-007	CY2026 UDS change log 2026	Regulatory tracking evidence (PAL 2025-05)	Compliance Lead	2026-03-15	Complete

Open Tasks

Task	Owner	Due Date	Priority	Status
Upload BP retraining sign-in sheet	Nurse Manager	2026-04-05	High	Open
Finalize outreach script revision	PCMH Coordinator	2026-04-08	Medium	Open
Validate denominator logic for CRC list	QI Analyst	2026-04-10	High	In progress
Add committee approval note to SOP	Director of QI	2026-04-12	Low	Open

Every evidence item is traceable to a source file, document date, and named owner so an auditor or surveyor can verify recency and authenticity rather than relying on summary claims alone.

Leadership Review Snapshot

Committee Review

Meeting date: March 18, 2026 · Chair: Director of Quality Improvement

Participants: Medical Director, PCMH Coordinator, Nurse Manager, QI Analyst, and Clinical Operations Manager.

Agenda Highlights

Reviewed quarterly preventive screening trends; assessed progress of active PDSA cycles; confirmed documentation gaps requiring follow-up; reviewed HRSA PAL 2025-05 CY2026 UDS proposed changes and assigned readiness tasks; approved outreach workflow revisions for the next test cycle.

Decisions Recorded

Continue CRC outreach with revised reminder cadence; adopt BP recheck workflow with additional float staff training; maintain weekly evidence review until all Q1 artifacts are complete; assign Compliance Lead to monitor final 2026 UDS Manual release.

Open Actions From This Review

Action	Owner	Due
Upload BP retraining roster	Nurse Manager	2026-04-05
Validate CRC denominator logic	QI Analyst	2026-04-10
Confirm 2026 UDS Manual release and reconcile against PAL 2025-05 tracker	Compliance Lead	2026-05-01

This review creates a visible audit trail showing what leadership reviewed, what decisions were made, and which follow-up actions remain owned — concise enough for executives, specific enough for compliance review, and traceable enough for follow-through.

CY2026 UDS Reporting Alignment

NEW

On December 8, 2025, HRSA issued **Program Assistance Letter (PAL) 2025-05** outlining proposed changes to the calendar year 2026 Uniform Data System (UDS), to be reported by health centers in February 2027. MeasureWise maintains a living crosswalk between current UDS measures/tables and each year's proposed HRSA changes so quality teams can plan documentation, EHR mapping, and workflow updates ahead of the final 2026 UDS Manual.

Why this matters for audit readiness

Several proposed 2026 changes affect measures this binder already tracks (Colorectal Cancer Screening, Hypertension Control) and introduce new reportable populations (Type 1 diabetes, IDD, ASD screening, patient support services). Confirming readiness now avoids scrambling when the final Manual is released.

eCQM Version Updates Affecting Tracked Measures

Measure in this Binder	2025 eCQM	2026 eCQM (Proposed)	Status
Colorectal Cancer Screening	CMS130v13	CMS130v14	Tracked
Cervical Cancer Screening	CMS124v13	CMS124v14	Tracked
Controlling High Blood Pressure	CMS165v13	CMS165v14	Tracked
Diabetes: Glycemic Status Assessment >9%	CMS122v13	CMS122v14	Tracked

Full list of 14 eCQMs updated for 2026 (per PAL 2025-05 Attachment 4) is maintained in the MeasureWise regulatory library, including Depression Remission (CMS159 v14 → v15), Screening for Depression and Follow-Up (CMS2 v14 → v15), Statin Therapy for CVD (CMS347 v8 → v9), and HIV Screening (CMS349 v7 → v8, with updated denominator logic).

New CY2026 Table 6A Measures to Plan For

New Measure	Line	Rationale (per HRSA)
Diabetes Mellitus Type 1	9a	Distinguishes Type 1 from aggregated diabetes reporting to identify distinct care/resource needs
Intellectual and Developmental Disabilities (IDD)	20g	Addresses data gaps for a population with lower preventive/chronic care access
Autism Spectrum Disorder (ASD) Screening	26g	Strengthens early childhood development data and screening practice visibility
Patient Support Services (case mgmt, eligibility, transportation, language)	35–38	Captures non-clinical services connected to access and outcomes
Health-Related Needs Services (screening, food, housing, financial insecurity)	39–42	Elevates SDOH screening/response data into the UDS core tables

Reporting Table Restructuring

Table	Proposed Change
Table 4	Managed care member months (Lines 13a–13c) removed to reduce burden
Table 5	“Enabling Services” renamed to “Patient Support Services”; QI personnel line moved under IT personnel
Table 6B	Dental Sealants (children 6–9) measure replaced with Sealant Receipt on Permanent First Molars (SFM-CH), aligning to CMS Core Set
Table 8A	Cost columns restructured to Personnel (a1) / Other Costs (a2) / Total Accrued Costs (a); Quality Improvement and Enabling Services detail lines consolidated
Table 9D	Shifts from cash to accrual basis; adds Net Patient Service Revenue, Pharmacy Revenue, and Third-Party Incentive Revenue lines
Table 9E	Shifts to accrual basis; BPHC and other federal grant detail lines aggregated to totals
Appendix D & E	Consolidated into one appendix; adds 3 new Alternative Payment Model (APM) questions; several legacy Health IT/COVID-era questions removed

Source: HRSA PAL 2025-05, “Proposed Uniform Data System Changes for Calendar Year 2026,” issued December 8, 2025. These are proposed changes; final specifications will be published in the 2026 UDS Manual.

MeasureWise Readiness Checklist for CY2026

Readiness Item	Status	Notes
PAL 2025-05 change log documented	Ready	Logged as evidence item EV-007
Tracked eCQMs cross-walked to 2026 versions	Ready	See eCQM table, this section
New Table 6A measures scoped for EHR/workflow build	Needs follow-up	IDD and ASD screening capture not yet configured
Table 8A/9D/9E accrual-basis transition reviewed with finance	Not started	Owner to be assigned at next QI committee meeting
Final 2026 UDS Manual monitored for confirmation	Ongoing	Compliance Lead subscribed to HRSA BPHC updates

Audit Readiness View

Requirement Area	Status	Notes
Written QI activity documented	Ready	PDSA logs included, dated and owned
Measure review documented	Ready	Quarterly trend summary with baseline/target included
Leadership oversight visible	Ready	Committee review documented with date and attendees
Assigned accountability visible	Ready	Owners listed across all tasks and projects
Follow-up actions tracked	Needs follow-up	Two high-priority open items; both have owners and due dates
Supporting evidence organized	Ready	Evidence register maintained with source and document dates
CY2026 UDS change readiness	NEW Needs follow-up	2 of 5 readiness items open — see Section 9

Overall Readiness: Needs Follow-up

6 of 7 evidence areas are ready; 2 areas have open, owned, dated action items pending closure, including CY2026 UDS transition planning. No area is currently at risk or unowned.

What a Live Export Can Include

Live export capabilities

Date-filtered histories, site comparisons, trend visuals, attached source files, appendices, and print-ready review sections.

Export metadata

Reporting period, generation date, filters applied (organization/site/measure/project), preparer, document version, and applicable UDS reporting year.

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